

Pacific General Hospital — Discharge Medication Reconciliation

Patient: **Nora Cohen** · MRN 0000123488 · DOB 06/04/1967 · Discharged 04/22/2026 1530 from 4N (Ortho-Med) ·
Discharging hospitalist: Jorge Reyes, MD

CONTINUED — no changes from home regimen

Medication	Dose	Route	Frequency	Indication	Prescriber
Levothyroxine	112 mcg	PO	Daily, empty stomach	Hypothyroidism	Park, D.
Sertraline	100 mg	PO	Daily AM	Recurrent MDD	Jensen, M.
Calcium Carbonate + Vitamin D3	1200 mg/2000 IU	PO	Daily with food	Bone health	OTC

CHANGED — switched agent

Medication	Dose	Route	Frequency	Indication	Prescriber
Zoledronic Acid (Reclast)	5 mg	IV	Once yearly (next due 05/2027)	Osteoporosis (replacing oral Alendronate after vertebral fx)	Reyes, J.

NEW — started during this admission

Medication	Dose	Route	Frequency	Indication	Prescriber
Acetaminophen	650 mg	PO	Q6H scheduled x 14 days, then PRN	Vertebral Pain	Reyes, J.
Lidocaine 5% Patch	1 patch	TOP	Apply to mid-back x 12 hours, 12h off	Localized pain	Reyes, J.
Polyethylene Glycol	17 g	PO	Daily PRN constipation	Bowel reg with rest	Reyes, J.

STOPPED — do not continue at home

Medication	Dose	Route	Frequency	Indication	Prescriber
Alendronate (Fosamax)	70 mg	PO	Was once-weekly	Switched to IV zoledronic (Reclast).	Park, D.

Patient verbalized understanding of all medication changes and was provided a written copy. Hold ibuprofen and other NSAIDs for at least 6 weeks while the L2 compression fracture is healing. Follow-up with primary care (Dr. Jensen) within 7 days; endocrinology (Dr. Park) within 30 days for repeat DEXA planning and Reclast counseling.

Reconciled by: Marisol Ortega, RN · Reviewed by: Jorge Reyes, MD · 04/22/2026 1452